SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

Capecitabine + radiotherapy

Locally advanced rectal cancer · NCCP 00586 · version 3 · 7-day cycle

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Patient name	DOB	Hospital no.	Date

Treatment context
Capecitabine tablets are given with pelvic radiotherapy for locally advanced rectal cancer. The tablets help the radiation treatment work; your team confirms how this fits with surgery and other treatment.

Schedule
Capecitabine twice daily on Days 1–5 of each 7-day cycle, alongside radiotherapy, for up to 5–6 cycles. Follow the written plan for treatment days and interruptions.

1. GENERIC SACT RISKS / DISCUSSION POINTS

- ☐ Purpose, expected benefit and treatment plan explained
- ☐ Treatment may be delayed, modified or stopped because of toxicity
- ☐ Unexpected, rare or serious adverse effects can occur
- ☐ Supportive medicines and urgent contact advice discussed
- ☐ Pregnancy avoidance / contraception / fertility discussed where relevant
- ☐ Bone-marrow suppression / infection risk
- ☐ Anaemia / fatigue and low platelets / bleeding
- ☐ Nausea / vomiting and bowel disturbance
- ☐ Mouth soreness / mucositis and skin changes
- ☐ Allergic / infusion reaction and other organ-specific toxicity

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Capecitabine
ORAL CHEMOTHERAPY

- ☐ Diarrhoea and dehydration
- ☐ Painful hands / feet
- ☐ Mouth soreness and reduced intake
- ☐ Low blood counts / serious infection
- ☐ Chest pain / heart problems
- ☐ Severe early toxicity, including DPD-related toxicity

Pelvic radiotherapy
LOCAL RADIATION TREATMENT

- ☐ Bowel irritation / diarrhoea
- ☐ Pelvic skin soreness
- ☐ Urinary symptoms
- ☐ Fatigue
- ☐ Possible lasting bowel / bladder effects
- ☐ Sexual function / fertility effects

Confirm DPD testing, tablet directions, missed-dose and stop-and-call instructions, kidney/liver and blood monitoring, interactions, fertility and contraception. Discuss radiotherapy-specific early and late effects and the separate radiation consent process.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00586 v3.

SACTCheck v0.83.4
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